



WEEKLY EPIDEMIOLOGICAL REPORT

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Journey to malaria elimination and challenges in sustaining Prevention of Re-establishment (PoR) phase of malaria in Sri Lanka - III

This is the third article of three in a series on "Journey to malaria elimination and challenges in sustaining Prevention of Re-establishment (PoR) phase of malaria in Sri Lanka"

- **Surveillance and diagnostics** — suspected cases are notified, investigated, and responded to within five days, using quality-assured RDTs, microscopy, and PCR. Regular cross-validation and refresher training keep laboratory accuracy high, while the AMC's role as sole importer of WHO-prequalified medicines and diagnostics protects the supply chain from counterfeits.
- **Border health and targeted screening** — since 2016, active and passive case detection at healthcare facilities and points of entry has been paired with migration surveillance and pre-travel chemoprophylaxis for high-risk groups (migrants, refugees, military, outdoor workers). Specific outreach extends to gem traders, seafarers, pilgrims, and UN peacekeepers, offering free chemoprophylaxis, health education, and post-arrival screening.
- **Governance** — The Technical Support Group (established 2012), combining independent experts and Ministry of Health representatives, classifies every case, guides strategy, advocates for funding, and supports research. Despite health devolution under the Thirteenth Amendment, the AMC has preserved centralized technical oversight of Regional Malaria Officers, enabling standardized, rapid national responses.
- **Vector control** — Indoor residual spraying, insecticidal nets, larval source management, entomological surveillance, and environmental management remain core activities, reinforced by a 2024 WHO-supported review that strengthened surveillance and cross-border collaboration recommendations.

risk is intensified by several collecting factors (13):

- * **Mobility** — labor migration, tourism, seafaring, and cross-border travel are the main routes for imported cases, especially where post-arrival screening is inconsistent.
- * **Climate change** — shifting temperature and rainfall patterns, plus urban-adapted vectors like *Anopheles stephensi*, could expand transmission risk into new areas.
- * **Resistance trends** — emerging artemisinin resistance and weakening diagnostic test sensitivity threaten early detection.
- * **Eroding expertise** — as malaria becomes rarer, clinical suspicion and laboratory microscopy skills decline, risking delayed diagnosis.
- * **Complacency risk** — past success can reduce funding and political urgency, and Sri Lanka's history shows premature relaxation causes resurgence.
- * **Fragmented coordination** — effective prevention requires tight collaboration across health, immigration, tourism, defense, and local government, which can be difficult to sustain.

Conclusion and the Way Forward

Sri Lanka's malaria-free status must be actively defended, against climate change, human mobility, drug resistance, and fading institutional expertise(14). It demands the same vigilance that achieved elimination, sustained indefinitely rather than treated as finished (15). Continued PoR investment is best understood as insurance against a far greater future loss, not a recurring expense. The way forward stand on four mutually reinforcing pillars(16):

Challenges to Sustaining Zero Transmission

Sri Lanka's greatest ongoing risk is **imported malaria** entering a still-receptive environment where local vectors could allow reintroduced cases to spark new transmission(11)(12). This

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Strengthened surveillance	• 24-hour mandatory case notification, real-time case investigation, and continuous vector monitoring.
Robust border health measures	• Targeted screening, chemoprophylaxis, and full implementation of the National Migration Health Policy
Programme integration and modernization	• Linking malaria surveillance with other vector-borne disease programmes such as dengue, alongside digital reporting and regional data-sharing
Preserved technical leadership	• Maintaining the Anti-Malaria Campaign's centralized expertise and continuous diagnostic training, rather than diluting it through premature institutional integration

Sustaining these pillars will require unwavering political commitment, adequate domestic financing, and the flexibility to adapt as global travel, migration, climate variability, and emerging biological threats evolve. It will also mean resisting the two temptations that have undone progress before: the complacency that followed the near-elimination of 1963, and the premature institutional integration that slowed leprosy control. Deepening collaboration across health, tourism, defense, immigration, and labor sectors remains equally essential to monitor and manage risk jointly(17).

By preserving evidence-based policy, technical expertise, and resilient health systems, Sri Lanka can safeguard its malaria-free status, prevent resurgence, and continue to serve as a global blueprint for the sustainable prevention of malaria re-establishment.

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Table 1: Distribution of Notified Diseases reported by Medical Officers of Health 20th – 26th July 2026 (30th Week)

RDHS	Dengue Fever		Dysentery		Encephalitis		En. Fever		F. Poison-		Leptospirosis		Typhus		Viral Hep.		H. Rabies		Chickenpox		Meningitis		Leishman.		Tuberculosis		Leprosy		WRCD	
	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	T*	C**
Colombo	1374	16088	1	17	0	2	0	4	0	19	6	205	0	2	0	12	0	0	11	355	1	68	1	4	32	1076	1	99	100	100
Gampaha	1320	13586	2	32	0	30	0	0	0	23	13	340	0	5	0	9	0	0	9	555	16	200	3	39	23	603	0	47	86	100
Kalutara	499	6469	0	34	0	3	1	9	0	62	6	280	1	7	0	17	0	0	20	556	0	92	0	12	9	346	4	58	80	100
Kandy	567	5431	0	39	0	8	0	7	0	38	8	143	0	26	0	18	0	0	15	449	4	99	0	41	5	307	0	19	60	100
Matale	70	2063	0	19	0	5	0	2	0	7	2	144	0	3	0	8	0	0	4	142	2	57	12	328	6	107	0	24	75	100
Nuwara Eliya	78	913	1	35	0	5	0	3	0	15	5	210	2	34	0	13	0	0	14	340	1	146	1	2	5	152	1	9	93	100
Galle	308	5373	1	24	0	11	0	8	1	56	9	381	1	30	1	18	0	0	22	682	11	121	0	2	7	253	0	38	73	93
Hambantota	122	2391	1	38	0	2	0	0	0	39	8	173	0	22	0	11	0	0	2	157	2	47	9	320	3	86	0	21	100	100
Matara	380	5772	0	18	0	3	0	2	0	11	11	266	0	17	0	15	0	0	4	378	3	134	4	96	5	94	2	16	98	100
Jaffna	35	919	3	60	0	6	0	19	0	90	1	53	2	231	0	1	0	0	5	268	0	37	0	1	10	113	0	11	84	100
Kilinochchi	8	635	0	12	0	1	0	8	0	1	0	76	0	12	0	3	0	1	1	92	0	12	0	2	0	20	0	2	100	100
Mannar	4	527	0	1	0	3	0	0	0	3	0	45	0	4	0	3	0	0	0	63	0	20	0	5	0	29	0	3	100	100
Vavuniya	16	312	0	19	0	4	0	2	0	8	0	98	0	4	0	1	0	0	3	117	0	30	0	21	0	42	1	7	100	100
Mullaitivu	8	86	0	6	0	2	0	1	0	8	0	34	0	4	0	2	0	0	0	11	0	9	1	10	1	18	0	7	100	100
Batticaloa	56	1400	1	55	0	10	0	2	5	21	4	141	0	2	0	16	0	0	5	217	1	70	1	22	3	93	4	73	100	100
Ampara	36	547	2	50	0	1	0	1	0	16	3	115	0	2	0	8	0	0	4	238	3	59	0	13	1	31	2	19	93	100
Trincomalee	23	691	0	18	0	6	0	2	0	14	1	75	0	8	0	2	0	0	6	156	1	28	0	18	0	93	0	6	100	100
Kurunegala	235	2943	1	19	0	16	0	3	0	65	6	292	1	26	0	10	0	1	15	550	10	283	13	284	7	194	1	47	94	100
Puttalam	171	1512	0	16	0	11	0	0	2	9	1	186	1	22	0	7	0	4	5	112	6	123	0	15	0	107	1	19	77	100
Anuradhapura	89	895	0	18	0	8	0	1	0	49	2	184	0	22	0	8	0	0	12	393	1	117	24	493	6	159	2	37	90	100
Polonnaruwa	53	719	2	23	0	7	0	2	0	59	3	220	0	7	0	24	0	0	15	336	0	51	12	357	4	54	1	58	100	100
Badulla	174	1420	0	32	0	8	0	3	0	13	6	186	4	37	3	112	0	0	12	304	3	200	2	86	3	155	0	15	95	100
Monaragala	74	1104	0	14	0	3	0	1	0	8	4	206	0	28	1	42	0	1	5	167	1	76	5	148	2	74	1	14	96	100
Ratnapura	302	4742	1	26	0	6	0	6	1	18	13	636	0	29	1	12	0	0	5	268	5	75	7	139	10	251	1	25	93	99
Kegalle	201	2334	0	41	1	5	0	4	0	24	12	278	0	10	0	12	0	0	20	474	5	72	0	13	5	191	0	7	58	100
Kalmunai	43	892	3	39	1	2	0	1	0	19	4	60	0	1	0	3	0	0	5	357	7	33	0	0	0	73	2	25	98	100
SRILANKA	6246	79764	19	705	2	168	1	91	9	695	128	5027	12	595	6	387	0	7	219	7737	83	2259	95	2471	147	4721	24	706	90	100

Source: WRCD module of the EPINET. T*=Timeliness refers to returns received on or before 26th July, 2026. C**=Completeness; A = Cases reported during the current week; B = Cumulative cases for the year. Total number of reporting units 360 C**=Completeness;

Table 2: Selected Vaccine Preventable Diseases & AFP

20th – 26th July 2026 (30th Week)

Disease	No. of Cases by Province									Number of cases during current week in 2026	Number of cases during same week in 2025	Total number of cases to date in 2026	Total number of cases to date in 2025	Difference between the number of cases to date in 2026 & 2025
	W	C	S	N	E	NW	NC	U	Sab					
AFP ¹	00	00	00	00	01	00	00	00	00	01	02	42	36	16.6%
Diphtheria	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Mumps ²	00	01	00	00	00	02	00	00	00	03	03	87	95	-8.4 %
Measles ³	00	01	01	00	00	00	00	00	03	05	00	39	02	1850 %
Rubella ³	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
CRS ²	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Tetanus ²	00	00	00	00	00	00	00	00	00	00	00	04	03	33.3 %
Neonatal Tetanus ²	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Japanese Encephalitis ³	00	00	00	00	00	00	00	00	00	00	01	00	04	-100 %
Whooping Cough ²	00	00	00	00	00	00	00	00	00	00	00	19	12	58.3 %

Key to Table 2

Provinces: W: Western, C: Central, S: Southern, N: North, E: East, NC: North Central, NW: North Western, U: Uva, Sab: Sabaragamuwa.

Data Sources:

Weekly Return of Communicable Diseases: Diphtheria, Mumps, Tetanus, Neonatal Tetanus, Whooping Cough.

Special Surveillance: AFP, Measles, Rubella, CRS.

AFP¹ = No Polio cases

Mumps², CRS², Tetanus², Neonatal Tetanus², Whooping Cough²—Clinically and/ or laboratory confirmed cases

Measles³, Rubella³, Japanese Encephalitis³— Laboratory Confirmed cases

AFP—Acute Flaccid Paralysis

CRS = Congenital Rubella Syndrome

NA = Not Available

AFP and all Vaccine Preventable Diseases should be investigated by the MOH Personally.

All patients presenting with fever and a maculopapular rash must be promptly notified and investigated. Blood samples and throat swabs should be collected at the appropriate time window

Comments and contributions for publication in the WER Sri Lanka are welcome. However, the editor reserves the right to accept or reject items for publication. All correspondence should be mailed to The Editor, WER Sri Lanka, Epidemiology Unit, P.O. Box 1567, Colombo or sent by E-mail to chepid@sltnet.lk. The Epidemiology Unit should be formally acknowledged in all resulting publications as the primary data source.

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